ADULT CARDIAC SURGERY

GUIDELINES 2025
EUROPEAN RESUSCITATION COUNCIL©

1. Prevent & be prepared



Ensure adequate training of the staff in technical skills and ALS



Ensure availability and functioning of emergency equipment

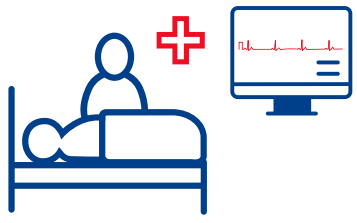


Use safety checklists



Identify and manage deterioration in the post-operative cardiac patient

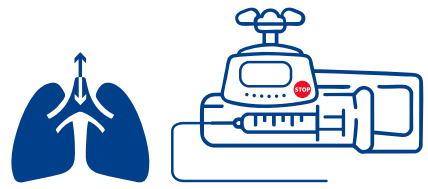
2. Detect cardiac arrest and activate cardiac arrest protocol



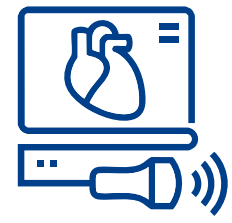
Confirm cardiac arrest by clinical signs and pulseless waveforms



Call for help and activate cardiac arrest protocol



- Ensure airway patency and breathing
- Deliver 100% oxygen
- Stop syringe drivers



Consider echocardiography to early identify possible reversible causes

3. Resuscitate and treat possible reversible causes

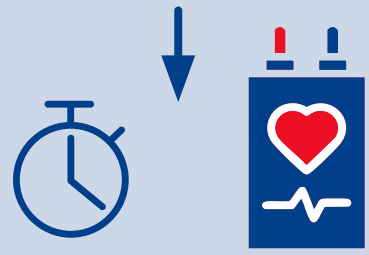
ASSESS RHYTHM

VF/pVT



Defibrillate
(apply up to 3 consecutive shocks)

Asystole / extreme bradycardia



Pace -
if wires are available

PEA



Provide high-quality CPR

Start basic life support and address reversible causes

Amiodarone 300 mg IV	Consider external pacing if wires are not available	If paced, turn off pacing to exclude underlying VF

No ROSC

- Continue advanced life support
- Prepare & perform early resternotomy (<5 min)
- Consider circulatory support devices and ECPR

ALS – advanced life support; CPR - cardiopulmonary resuscitation; ECPR - extracorporeal cardiopulmonary resuscitation; VF - ventricular fibrillation; pVT - pulseless ventricular tachycardia; ROSC – return of spontaneous circulation; PEA - pulseless electrical activity; IV - intravenous